

ICU TO PALLIATIVE CARE

Transition Documentation Framework

Nursing & Multidisciplinary Team Record

IMPORTANT: This framework is a supplementary documentation tool. It does NOT replace the existing Withholding & Withdrawing Treatment documentation. Cross-reference those documents where applicable. For Medical Board processes, refer to institutional policy.

PATIENT INFORMATION

Study ID / MRN	ICU Location / Bed	Date of ICU Admission
Age	Sex ☐ Male ☐ Female ☐ Other	Date of PC Referral

Primary Diagnosis

Reason for ICU Admission

Prognosis (brief summary)

Attending ICU Consultant

Palliative Care Team / Consultant

Administrative Notes / Cross-reference Document Numbers (e.g. W&W form ref, MRN, episode number)

PHASE 1 IDENTIFICATION OF PALLIATIVE CARE NEED

Was the patient identified as having serious health-related suffering requiring palliative care input?

- ☐ Yes — proceed to Phase 2 ☐ No — document reason below ☐ Under review / pending

Apt Care ICU framework – pilot work in progress (Attached an annexure 7)

- | | |
|--|---|
| ☐ Poor prognosis / life-limiting illness | ☐ Multiple organ failure (≥ 2 organs) |
| ☐ Advanced malignancy | ☐ Prolonged mechanical ventilation (> 7 days) |
| ☐ Repeated ICU admission (same illness episode) | ☐ Severe, refractory symptom burden |
| ☐ Functional decline — unable to perform ADLs | ☐ Futility of ongoing curative treatment |
| ☐ Family / surrogate request | ☐ Senior clinician concern |
| ☐ Patient expressed wishes / advance directive | ☐ Ethical concern raised by team or family |
| ☐ Other (specify in notes below) | |

Identification Timeline

Initial date of ICU non-beneficence considered

Time from ICU admission to identification

Staff member(s) who identified / raised concern (name, designation)

Barriers to Early Identification

- | | |
|--|---|
| ☐ Prognostic uncertainty | ☐ Time-limited trial of an intervention ongoing |
| ☐ Lack of ICU team consensus | ☐ Insufficient awareness of scope of palliative care |
| ☐ No formal screening framework/ personnel in use | ☐ Patient / family not ready |
| ☐ Communication difficulties (language, culture) | ☐ Other (specify in notes) |

What delayed or prevented identification? (describe factors, context, clinical reasoning)

Phase 1 — Additional Notes / Observations

PHASE 2 REFERRAL TO PALLIATIVE CARE
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Referral date	Referral time
Referring clinician (name, designation) Accepting PC clinician / team	Reasons for Referral
Time from decision of non-beneficence, to PC referral	Time from admission to PC referral

Referral Pathway

- | | |
|--|--|
| ☐ Formal written referral | ☐ Telephone / verbal referral / posting in group (to be formalised) |
| ☐ Direct PC team involvement (embedded model) | ☐ Ethics committee / board involvement |

Reasons for Referral Delay

- | | |
|--|--|
| ☐ Weekend / public holiday | ☐ PC consultant unavailable |
| ☐ Prognostic uncertainty — team not ready | ☐ Family not yet ready |
| ☐ No ICU team consensus on goals | ☐ Awaiting further investigation results |
| ☐ Administrative / system delay | ☐ Competing clinical priorities |
| ☐ Cross-cultural or language barrier | ☐ Belief that patient 'not suitable' for PC |
| ☐ Other (specify) | |

Communication at Referral

Communication Item	Details / Notes
☐ Handover information conveyed to PC team	
☐ Copy of W&W documentation shared	
☐ Nursing handover to PC nurse	
☐ Family informed of referral	

Facilitators — what helped the referral process?

Barriers — what made referral difficult?

Phase 2 — Additional Notes / Referral Context

PHASE 3 CLINICAL ASSESSMENT

<i>Complete a structured symptom assessment on first PC contact. Use validated tools where available (e.g. IPOS, Edmonton Symptom Assessment System). Document severity rating where possible (0–10 scale).</i>

Symptom Assessment

Symptom Domain	Present	Severity (0–10)	Intervention / Management Plan
Pain	☐ Y ☐ N		
Breathlessness / Dyspnoea	☐ Y ☐ N		
Delirium / Agitation	☐ Y ☐ N		
Anxiety / Psychological distress	☐ Y ☐ N		
Nausea / Vomiting	☐ Y ☐ N		
Fatigue	☐ Y ☐ N		
Nutritional / Hydration needs	☐ Y ☐ N		
Pressure injury / Wound care	☐ Y ☐ N		
Oral care needs	☐ Y ☐ N		
Bladder / Bowel symptoms	☐ Y ☐ N		
Spiritual / Existential distress	☐ Y ☐ N		
Family distress / Carer needs	☐ Y ☐ N		
Functional decline / Mobility	☐ Y ☐ N		

Assessment Documentation

- | | |
|--|--|
| ☐ Validated tool used (specify below) | ☐ Functional status documented (e.g. PPS, ECOG) |
| ☐ Psychological assessment completed | ☐ Social / family assessment completed |
| ☐ Spiritual assessment completed | ☐ Advance care planning discussed |

Assessment tool(s) used

Facilitators for assessment

Barriers to comprehensive assessment (e.g. patient unable to communicate, language barrier, acute clinical deterioration)
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Phase 3 — Clinical Notes

PHASE 4 FAMILY / SURROGATE MEETING

Document each meeting separately if multiple meetings occur. Where patient has capacity, document patient involvement directly.

Meeting Details

Date of meeting	Time
Duration	Location

Meeting conducted?

- ☐ Yes ☐ No — document reason below
- ☐ Attempted — family unavailable / declined ☐ Patient had capacity — direct discussion

If no meeting — reason and plan for rescheduling

Participants (name & relationship / designation)

Topics Discussed

- | | |
|---|--|
| ☐ Current clinical condition and prognosis | ☐ Diagnosis and illness trajectory |
| ☐ Treatment options — benefits and burdens | ☐ Goals of care discussion |
| ☐ Withholding / withdrawing treatment (cross-ref W&W form) | ☐ CPR status |
| ☐ Symptom management plan | ☐ Transfer / discharge options |
| ☐ Home care / hospice | ☐ Spiritual / cultural needs |
| ☐ Interpreter / cultural support provided | ☐ Patient's previously expressed wishes |
| ☐ Advance care directive reviewed | ☐ Legal next-of-kin / guardianship |

Communication Quality

Communication Item	Yes	No / Partial	Notes
Interpreter provided (if needed)	☐	☐	
Plain language used	☐	☐	
Written information given to family	☐	☐	
Time allowed for questions	☐	☐	
Emotional support offered during meeting	☐	☐	
Family understanding confirmed	☐	☐	

Communication Breakdown / Concerns

- ☐ Family disagreement with clinical team ☐ Disagreement between family members

- | | |
|--|--|
| ☐ Language / cultural barrier | ☐ Conflicting information given by different clinicians |
| ☐ Family in denial regarding prognosis | ☐ Information delivered insensitively |
| ☐ Family felt excluded from decision-making | ☐ Patient's wishes not explored or documented |
| ☐ Surrogacy / legal decision-making unclear | ☐ None identified |

Describe any communication breakdown and how it was / will be addressed

Family Understanding & Decision
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Family understanding of prognosis:

- | | |
|--|---|
| ☐ Good — full understanding | ☐ Partial — some gaps remain |
| ☐ Poor — significant misunderstanding | ☐ Unable to assess |

Was a decision reached?

- | | |
|--|---|
| ☐ Yes — documented in goals of care (Phase 5) | ☐ No — further discussion needed |
| ☐ Decision deferred pending Medical Board | |

Summary of decision / family position
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Facilitators

Barriers

Phase 4 — Meeting Notes

PHASE 5 GOALS OF CARE DOCUMENTATION

Goals of care should align with and cross-reference the Withholding & Withdrawing Treatment document. Ensure documentation is consistent across all records.

Goals of care documented in the medical record?

- ☐ Yes — complete ☐ Partial — gaps remain (document below) ☐ No — pending (document reason)

Goals of Care Type

- ☐ Full treatment — all life-sustaining therapies ☐ Time-limited trial — reassess by [date]
☐ Comfort-focused — symptom management priority ☐ DNR / DNAR (cross-ref W&W form)
☐ Withdrawal of specified treatment (cross-ref W&W form) ☐ Hospice / specialist palliative care model of care

Date goals documented

Time-limited trial review date (if applicable)

Documentation Completeness Check

Documentation Item	Complete	Partial / Absent	Reference / Notes
Quality department approved Goals of care document	☐	☐	
Goals of care documented in medical notes	☐	☐	
Goals communicated to nursing staff	☐	☐	
Goals communicated to allied health	☐	☐	
W&W form completed and cross-referenced	☐	☐	
Patient / family copy provided	☐	☐	
Goals documented in nursing care plan	☐	☐	
Medication review completed (ceasing unnecessary drugs)	☐	☐	
Resuscitation status clearly documented	☐	☐	

Documentation Delays

- ☐ Medical officer unavailable to document ☐ Disagreement between team members
☐ Awaiting family / surrogate decision ☐ Awaiting Medical Board outcome
☐ Complexity of case — documentation took time ☐ IT / system issues
☐ Competing clinical demands ☐ Other (specify in notes)

Time from goals of care decision to documentation (estimated)

Facilitators for goals of care documentation

Barriers to goals of care documentation

Phase 5 — Additional Notes

PHASE 6 MEDICAL BOARD PROCESS (IF APPLICABLE)

Complete this phase only if a Medical Board was convened. Cross-reference institutional policy and existing W&W documentation.

Was a Medical Board required?

☐ Yes ☐ No — proceed to Phase 7 ☐ Under consideration

Setting/ Department:

ICU / IP ward

Coordinated by:

Primary Medical Board

Date convened	Time
Board members present (designation)	Quorum met? ☐ Yes ☐ No

Primary Medical Board completed?

☐ Yes — outcome documented ☐ No — adjourned (reason below) ☐ Not required

Primary Board outcome / recommendation

Secondary Medical Board (if required)

Date convened	Time required from primary to secondary board
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Secondary Medical Board completed?

☐ Yes — outcome documented ☐ No — adjourned ☐ Not required

Secondary Board outcome / recommendation

Board Documentation & Communication

☐ Board minutes completed	☐ Outcome documented in medical record
☐ Family informed of board outcome	☐ Nursing team informed of outcome
☐ Outcome consistent with W&W documentation	☐ Legal / guardianship authority informed (if applicable)

Total time from referral to board completion

Facilitators for the Board process

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Barriers (e.g. quorum issues, delays, disagreements, documentation gaps)

Phase 6 — Board Notes

PHASE 7 ONGOING PALLIATIVE CARE
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Symptom Management

Care Domain	Adequate?	Actions / Plan
Pain management	☐ Y ☐ N ☐ NA	
Dyspnoea / breathlessness	☐ Y ☐ N ☐ NA	
Agitation / delirium	☐ Y ☐ N ☐ NA	
Nausea / comfort medications	☐ Y ☐ N ☐ NA	
Oral / mouth care	☐ Y ☐ N ☐ NA	
Skin / pressure care	☐ Y ☐ N ☐ NA	
Hydration / nutrition goals	☐ Y ☐ N ☐ NA	
Sleep / rest	☐ Y ☐ N ☐ NA	

Psychosocial & Spiritual Support

Support Domain	Provided?	Notes
Psychological support — patient	☐ Y ☐ N	
Psychological support — family / carers	☐ Y ☐ N	
Pastoral care / chaplaincy	☐ Y ☐ N	
Social work involvement	☐ Y ☐ N	
Cultural needs addressed	☐ Y ☐ N	
Bereavement support offered to family	☐ Y ☐ N	

Nursing Education & Team Preparation

- | | |
|---|--|
| ☐ Nursing staff educated on goals of care change | ☐ Comfort care nursing plan initiated |
| ☐ Medications reviewed — unnecessary treatments ceased | ☐ Monitoring adjusted to comfort-focused approach |
| ☐ Nursing handover to receiving unit completed | ☐ After-death care information provided (if applicable) |

Facilitators for ongoing care

Barriers to ongoing care (e.g. ICU environment, staffing, equipment)

Phase 7 — Ongoing Care Notes

Place of Death Documentation

Final Place of Death

- ☐ ICU
- ☐ Hospital Ward
- ☐ Palliative Care Unit
- ☐ Hospice
- ☐ Home
- ☐ Other: _____

Was the place of death aligned with the patient's/family's preferred place of care?

- ☐ Yes
- ☐ No
- ☐ Unknown

PHASE 8 TRANSITION / DISCHARGE**Transfer Destination**

- | | |
|---|--|
| ☐ General ward | ☐ Palliative care unit / ward |
| ☐ Hospice | ☐ Home (with community palliative care) |
| ☐ Home (family-led care, limited services) | ☐ Died in ICU |
| ☐ Other (specify in notes) | |

Transfer date	Transfer time
Receiving unit / facility	Receiving clinician / nurse
Total length of ICU stay	Time from PC referral to transfer

Discharge Planning Checklist

Task	Complete	N/A or Not Done	Responsible Person
Discharge summary completed	☐	☐	
Goals of care documented in discharge summary	☐	☐	
Medications reconciled and dispensed	☐	☐	
Equipment arranged (e.g. syringe driver, oxygen)	☐	☐	
Community referrals made (GP, community nurse, PC)	☐	☐	
Ambulance / transport arranged	☐	☐	
Family informed and prepared	☐	☐	
Family education on symptom management	☐	☐	
Emergency contact plan given to family	☐	☐	
After-hours / on-call plan documented	☐	☐	
Follow-up appointment arranged	☐	☐	
Interpreter / cultural support arranged for home	☐	☐	

Facilitators for transition

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Barriers to transition (e.g. bed availability, equipment, family readiness, community services)

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Phase 8 — Transition Notes

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OVERALL TRANSITION REFLECTION

Complete this section after the transition is finalised. This information will be used for quality improvement and service evaluation. Be specific and constructive.

Overall Assessment

Was this an appropriate and timely ICU → Palliative Care transition?

- ☐ Yes — timely and appropriate ☐ Partially — some delays or barriers
☐ No — significant delays / quality concerns ☐ Unable to determine

Overall time from ICU admission to transition (days)

What factors helped the transition? (describe specific enablers, people, processes, systems)

What made the transition difficult? (describe specific barriers, delays, communication issues, system gaps)

What could have been done earlier or better? (quality improvement perspective — clinical, communication, documentation, system)

Were there any communication breakdowns between the team, patient, or family? If so, describe and how they were managed.

EOL Care review details (Survey) – This data collection tool evaluates the quality of end-of-life care across the patient's illness trajectory. It captures the timeliness of end-of-life care recognition, communication with families, documentation of goals of care, and completion of legal documentation (CSI). It assesses symptom management, the use of invasive and life-sustaining treatments near the end of life, anticipatory prescribing, and the provision of holistic care across physical, emotional, social, and functional domains. The tool also documents the use of palliative sedation, including indications, documentation, medications, and duration. In addition, it evaluates grief assessment, bereavement support, logistical concerns following death, the quality of the death summary, and the healthcare team's experience in caring for the patient and family. Finally, it measures family perceptions of support, comfort, communication, and presence throughout the patient's journey.

Key Themes (for QI / audit purposes)

- ☐ Early identification could have occurred sooner ☐ Referral process needs improvement
☐ Documentation was incomplete or delayed ☐ Communication with family was a challenge
☐ Team communication or consensus was difficult ☐ Medical Board process added delay

- ☐ Discharge planning was inadequate ☐ Cultural / language needs not fully met
- ☐ Equipment / community services delayed transfer ☐ Positive model — could be used as learning case

Overall Reflections — Additional Notes / Recommendations for Service Improvement

Completed by: _____

Designation: _____

Date: _____

Signature: _____

This document is a supplementary ICU–Palliative Care transition record. It should be filed alongside existing Withholding & Withdrawing Treatment documentation and the medical record.

Post-ICU Transition to Palliative Care

Structured Follow-Up Framework: 48 Hours · 1 Week · 2 Weeks · 1 Month

How to use this form: Complete Section 1 at the time of transfer out of the ICU to establish a baseline assessment. The same assessment should then be completed at each of the four follow-up checkpoints (48 hours, 1 week, 2 weeks, 1 month) so that scores and concerns can be tracked consistently over time. Tick boxes where relevant; leave blank fields for free-text notes.

Section 1 — Patient Information (at ICU transfer)

Identification

Patient name	
Age / Sex	
Diagnosis	
Date of ICU transfer out	
Functional status (Karnofsky score)	

Baseline Documentation Status

Item	Y / N
Prognosis documented in records	☐ Y ☐ N
Advance care plan available	☐ Y ☐ N
DNR / withhold–withdraw discussion documented	☐ Y ☐ N
Palliative care symptom management needs identified	☐ Y ☐ N

Equipment / Transition Arrangements

☐ Home oxygen arranged
☐ Hospital bed/mattress arranged
☐ Suction machine arranged
☐ Feeding/catheter supplies arranged
☐ Other equipment arranged (specify below)

Other equipment / notes	
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Preferred Place of Care

☐ Home	☐ Palliative Care Ward
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☐ Hospice	☐ Long-term Care Facility
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Home Care Service Activation

☐ Not required	☐ Discussed with family
☐ Referred to outside agency	☐ Activated

Initial Family Education Provided (tick if covered)

☐ Bathing and hygiene care	☐ Oral/mouth care
☐ Skin care	☐ Pressure sore prevention/wound care
☐ Position changing/turning	☐ Eye care
☐ Hair and nail care	☐ Urinary catheter care
☐ Signs of catheter infection	☐ Feeding tube care (Ryle's/PEG)
☐ Tracheostomy care	☐ Suctioning technique
☐ Medication administration	

Section 2 — 48-Hour Follow-Up

Visit Details

Date of assessment	
Assessed by (name/role)	

Current Care Setting

☐ Home	☐ Palliative Care Ward
☐ Hospice	☐ Long-term Care Facility
☐ Hospital (general ward)	☐ Readmitted to ICU

Clinical Status

☐ Stable	☐ Improving
☐ Deteriorating	☐ Actively Dying

If Deceased

Date of death	
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Place of death:

☐ Home	☐ Hospital	☐ Hospice	☐ Other:
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Symptom Assessment — ESAS (score each 0–10)

Symptom	Score (0–10)
Pain	
Tiredness	
Drowsiness	
Nausea	
Appetite Loss	
Shortness of Breath	
Depression	
Anxiety	
Well-being	

Symptom	Score (0–10)
Restlessness/Agitation	
Delirium/Confusion	
Respiratory Distress	
Excessive Secretions	
Sleep Disturbance	
Thirst/Dry Mouth	
Pressure Injury	

Emotional Assessment

Emotional Assessment — Patient (if communicating)	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Fear of death					
☐ Unresolved fear related to ICU					
☐ Unresolved priority of being at home					
☐ Fear of suffering					
☐ Fear of being a burden					

☐ Loss of control					
☐ Hope for recovery					

<i>Emotional Assessment — Family</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Prognosis uncertainty					
☐ Financial concerns					
☐ Lack of insight of illness					
☐ Caregiver exhaustion					
☐ Spiritual concerns					
☐ Decision-making burden					
☐ Fear of death/suffering					
☐ Family conflict					
☐ Practical concerns (travel, accommodation)					
☐ Nursing care concerns					

Additional notes	
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Nursing Concerns Present

Item	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
Pain management issues					
Breathlessness management					
Delirium management					
Skin care concerns					
Nutrition concerns					
Communication difficulties					
Family distress observed					
End-of-life care needs					
Symptom monitoring difficulties					

Nursing Interventions Provided

☐ Positioning	☐ Skin care	☐ Mouth care
☐ Secretion management	☐ Pressure injury/wound care	☐ Catheter care
☐ Ryle's tube care	☐ Stoma care	☐ Tracheostomy care

Family Education — Reconfirm Competency

Tick if family demonstrates continued competency. If "Reinforcement needed", note the action taken below.

Education Topic	Competent	Reinforcement Needed
Bathing and hygiene care	☐	☐
Oral/mouth care	☐	☐
Skin care	☐	☐
Pressure sore prevention/wound care	☐	☐
Position changing/turning	☐	☐
Eye care	☐	☐
Hair and nail care	☐	☐
Urinary catheter care	☐	☐
Signs of catheter infection	☐	☐
Feeding tube care (Ryle's/PEG)	☐	☐
Tracheostomy care	☐	☐
Suctioning technique	☐	☐
Medication administration	☐	☐

Reinforcement notes	
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Advance Care Planning

☐ Goals of care documented and reviewed on daily basis
☐ CPR / resuscitation status discussed
☐ Future hospital admission plan discussed
☐ Preferred place of care confirmed
☐ Advance care plan updated in records

Home / Community Care Services

☐ Home visit completed	☐ Nursing visit completed
☐ Telephonic follow-up completed	☐ Equipment functioning
☐ Follow-up appointment attended	☐ Community nurse involved

Family Independently Managing

☐ Medications	☐ Catheter	☐ Feeding tube
☐ Oxygen	☐ Wound care	☐ Pressure sore prevention
☐ Comfort measures		

Psychosocial Status — Patient

☐ Emotionally stable	☐ Anxious
☐ Depressed	☐ Existential distress
☐ Not applicable	

Psychosocial Status — Family

☐ Coping well	☐ Caregiver burnout
☐ Anticipatory grief	☐ Bereavement needs identified

Action Plan / Next Steps

Issues identified	
Actions taken	
Next review date	
Reviewed by (name/role)	

Section 3 — 1-Week Follow-Up

Visit Details

Date of assessment	
Assessed by (name/role)	

Current Care Setting

☐ Home	☐ Palliative Care Ward
☐ Hospice	☐ Long-term Care Facility
☐ Hospital (general ward)	☐ Readmitted to ICU

Clinical Status

☐ Stable	☐ Improving
☐ Deteriorating	☐ Actively Dying

If Deceased

Date of death	
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Place of death:

☐ Home	☐ Hospital	☐ Hospice	☐ Other:
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Symptom Assessment — ESAS (score each 0–10)

Symptom	Score (0–10)
Pain	
Tiredness	
Drowsiness	
Nausea	
Appetite Loss	
Shortness of Breath	
Depression	
Anxiety	
Well-being	

Symptom	Score (0–10)
Restlessness/Agitation	
Delirium/Confusion	
Respiratory Distress	
Excessive Secretions	
Sleep Disturbance	
Thirst/Dry Mouth	
Pressure Injury	

Emotional Assessment

<i>Emotional Assessment — Patient (if communicating)</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Fear of death					
☐ Unresolved fear related to ICU					
☐ Unresolved priority of being at home					
☐ Fear of suffering					
☐ Fear of being a burden					
☐ Loss of control					
☐ Hope for recovery					

<i>Emotional Assessment — Family</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Prognosis uncertainty					
☐ Financial concerns					
☐ Lack of insight of illness					
☐ Caregiver exhaustion					
☐ Spiritual concerns					
☐ Decision-making burden					
☐ Fear of death/suffering					
☐ Family conflict					
☐ Practical concerns (travel, accommodation)					
☐ Nursing care concerns					

Additional notes	
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Nursing Concerns Present

Item	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
Pain management issues					
Breathlessness management					
Delirium management					
Skin care concerns					
Nutrition concerns					
Communication difficulties					
Family distress observed					
End-of-life care needs					
Symptom monitoring difficulties					

Nursing Interventions Provided

☐ Positioning	☐ Skin care	☐ Mouth care
☐ Secretion management	☐ Pressure injury/wound care	☐ Catheter care
☐ Ryle's tube care	☐ Stoma care	☐ Tracheostomy care

Family Education — Reconfirm Competency

Tick if family demonstrates continued competency. If "Reinforcement needed", note the action taken below.

Education Topic	Competent	Reinforcement Needed
Bathing and hygiene care	☐	☐
Oral/mouth care	☐	☐
Skin care	☐	☐
Pressure sore prevention/wound care	☐	☐
Position changing/turning	☐	☐
Eye care	☐	☐
Hair and nail care	☐	☐
Urinary catheter care	☐	☐
Signs of catheter infection	☐	☐
Feeding tube care (Ryle's/PEG)	☐	☐
Tracheostomy care	☐	☐
Suctioning technique	☐	☐
Medication administration	☐	☐

Reinforcement notes	
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Advance Care Planning

☐ Goals of care documented and reviewed
☐ CPR / resuscitation status discussed
☐ Future hospital admission plan discussed
☐ Preferred place of care confirmed
☐ Advance care plan updated in records

Home / Community Care Services

☐ Home visit completed	☐ Nursing visit completed
☐ Telephonic follow-up completed	☐ Equipment functioning
☐ Follow-up appointment attended	☐ Community nurse involved

Family Independently Managing

☐ Medications	☐ Catheter	☐ Feeding tube
☐ Oxygen	☐ Wound care	☐ Pressure sore prevention
☐ Comfort measures		

Psychosocial Status — Patient

☐ Emotionally stable	☐ Anxious
☐ Depressed	☐ Existential distress
☐ Not applicable	

Psychosocial Status — Family

☐ Coping well	☐ Caregiver burnout
☐ Anticipatory grief	☐ Bereavement needs identified
☐ Not Applicable	

Action Plan / Next Steps

Issues identified	
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Actions taken	
Next review date	
Reviewed by (name/role)	

Section 4 — 2-Week Follow-Up

Visit Details

Date of assessment	
Assessed by (name/role)	

Current Care Setting

☐ Home	☐ Palliative Care Ward
☐ Hospice	☐ Long-term Care Facility
☐ Hospital (general ward)	☐ Readmitted to ICU

Clinical Status

☐ Stable	☐ Improving
☐ Deteriorating	☐ Actively Dying

If Deceased

Date of death	
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Place of death:

☐ Home	☐ Hospital	☐ Hospice	☐ Other:
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Symptom Assessment — ESAS (score each 0–10)

Symptom	Score (0–10)
Pain	
Tiredness	
Drowsiness	
Nausea	
Appetite Loss	

Shortness of Breath	
Depression	
Anxiety	
Well-being	

Symptom	Score (0–10)
Restlessness/Agitation	
Delirium/Confusion	
Respiratory Distress	
Excessive Secretions	
Sleep Disturbance	
Thirst/Dry Mouth	
Pressure Injury	

Emotional Assessment

Emotional Assessment

<i>Emotional Assessment — Patient (if communicating)</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
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☐ Fear of being a burden					
☐ Loss of control					
☐ Hope for recovery					

<i>Emotional Assessment — Family</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Prognosis uncertainty					
☐ Financial concerns					
☐ Lack of insight of illness					

☐ Caregiver exhaustion					
☐ Spiritual concerns					
☐ Decision-making burden					
☐ Fear of death/suffering					
☐ Family conflict					
☐ Practical concerns (travel, accommodation)					
☐ Nursing care concerns					

Additional notes	
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Nursing Concerns Present

Item	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
Pain management issues					
Breathlessness management					
Delirium management					
Skin care concerns					
Nutrition concerns					
Communication difficulties					
Family distress observed					
End-of-life care needs					
Symptom monitoring difficulties					

Nursing Concerns Present

Item	Y / N
Pain management issues	☐ Y ☐ N
Breathlessness management	☐ Y ☐ N
Delirium management	☐ Y ☐ N
Skin care concerns	☐ Y ☐ N
Nutrition concerns	☐ Y ☐ N
Communication difficulties	☐ Y ☐ N

Family distress observed	☐ Y ☐ N
End-of-life care needs	☐ Y ☐ N
Symptom monitoring difficulties	☐ Y ☐ N

Nursing Interventions Provided

☐ Positioning	☐ Skin care	☐ Mouth care
☐ Secretion management	☐ Pressure injury/wound care	☐ Catheter care
☐ Ryle's tube care	☐ Stoma care	☐ Tracheostomy care

Family Education — Reconfirm Competency

Tick if family demonstrates continued competency. If "Reinforcement needed", note the action taken below.

Education Topic	Competent	Reinforcement Needed
Bathing and hygiene care	☐	☐
Oral/mouth care	☐	☐
Skin care	☐	☐
Pressure sore prevention/wound care	☐	☐
Position changing/turning	☐	☐
Eye care	☐	☐
Hair and nail care	☐	☐
Urinary catheter care	☐	☐
Signs of catheter infection	☐	☐
Feeding tube care (Ryle's/PEG)	☐	☐
Tracheostomy care	☐	☐
Suctioning technique	☐	☐

Reinforcement notes	
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Advance Care Planning

☐ Goals of care documented and reviewed
☐ CPR / resuscitation status discussed
☐ Future hospital admission plan discussed
☐ Preferred place of care confirmed

☐ Advance care plan updated in records

Home / Community Care Services

☐ Home visit completed	☐ Nursing visit completed
☐ Telephonic follow-up completed	☐ Equipment functioning
☐ Follow-up appointment attended	☐ Community nurse involved

Family Independently Managing

☐ Medications	☐ Catheter	☐ Feeding tube
☐ Oxygen	☐ Wound care	☐ Pressure sore prevention
☐ Comfort measures		

Psychosocial Status — Patient

☐ Emotionally stable	☐ Anxious
☐ Depressed	☐ Existential distress
☐ Not applicable	

Psychosocial Status — Family

☐ Coping well	☐ Caregiver burnout
☐ Anticipatory grief	☐ Bereavement needs identified
☐ Not applicable	

Action Plan / Next Steps

Issues identified	
Actions taken	
Next review date	
Reviewed by (name/role)	

Section 5 — 1-Month Follow-Up

Visit Details

Date of assessment	
Assessed by (name/role)	

Current Care Setting

☐ Home	☐ Palliative Care Ward
☐ Hospice	☐ Long-term Care Facility
☐ Hospital (general ward)	☐ Readmitted to ICU

Clinical Status

☐ Stable	☐ Improving
☐ Deteriorating	☐ Actively Dying

If Deceased

Date of death	
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Place of death:

☐ Home	☐ Hospital	☐ Hospice	☐ Other
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Symptom Assessment — ESAS (score each 0–10)

Symptom	Score (0–10)
Pain	
Tiredness	
Drowsiness	
Nausea	
Appetite Loss	
Shortness of Breath	
Depression	
Anxiety	
Well-being	

Symptom	Score (0–10)
Restlessness/Agitation	
Delirium/Confusion	
Respiratory Distress	

Excessive Secretions	
Sleep Disturbance	
Thirst/Dry Mouth	
Pressure Injury	

Emotional Assessment

<i>Emotional Assessment — Patient (if communicating)</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Fear of death					
☐ Unresolved fear related to ICU					
☐ Unresolved priority of being at home					
☐ Fear of suffering					
☐ Fear of being a burden					
☐ Loss of control					
☐ Hope for recovery					

<i>Emotional Assessment — Family</i>	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
☐ Prognosis uncertainty					
☐ Financial concerns					
☐ Lack of insight of illness					
☐ Caregiver exhaustion					
☐ Spiritual concerns					
☐ Decision-making burden					
☐ Fear of death/suffering					
☐ Family conflict					
☐ Practical concerns (travel, accommodation)					
☐ Nursing care concerns					

Additional notes	
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Nursing Concerns Present

Item	Being addressed	Partially Resolved	Resolved	Unresolved	Not Applicable
Pain management issues					
Breathlessness management					
Delirium management					
Skin care concerns					
Nutrition concerns					
Communication difficulties					
Family distress observed					
End-of-life care needs					
Symptom monitoring difficulties					

Nursing Concerns Present

Item	Y / N
Pain management issues	☐ Y ☐ N
Breathlessness management	☐ Y ☐ N
Delirium management	☐ Y ☐ N
Skin care concerns	☐ Y ☐ N
Nutrition concerns	☐ Y ☐ N
Communication difficulties	☐ Y ☐ N
Family distress observed	☐ Y ☐ N
End-of-life care needs	☐ Y ☐ N
Symptom monitoring difficulties	☐ Y ☐ N

Nursing Interventions Provided

☐ Positioning	☐ Skin care	☐ Mouth care
☐ Secretion management	☐ Pressure injury/wound care	☐ Catheter care
☐ Ryle's tube care	☐ Stoma care	☐ Tracheostomy care

Family Education — Reconfirm Competency

Tick if family demonstrates continued competency. If "Reinforcement needed", note the action taken below.

Education Topic	Competent	Reinforcement Needed
Bathing and hygiene care	☐	☐
Oral/mouth care	☐	☐
Skin care	☐	☐
Pressure sore prevention/wound care	☐	☐
Position changing/turning	☐	☐
Eye care	☐	☐
Hair and nail care	☐	☐
Urinary catheter care	☐	☐
Signs of catheter infection	☐	☐
Feeding tube care (Ryle's/PEG)	☐	☐
Tracheostomy care	☐	☐
Suctioning technique	☐	☐

Reinforcement notes	
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Advance Care Planning

☐ Goals of care reviewed
☐ CPR / resuscitation status discussed
☐ Future hospital admission plan discussed
☐ Preferred place of care confirmed
☐ Advance care plan updated in records

Home / Community Care Services

☐ Home visit completed	☐ Nursing visit completed
☐ Telephonic follow-up completed	☐ Equipment functioning
☐ Follow-up appointment attended	☐ Community nurse involved

Family Independently Managing

☐ Medications	☐ Catheter	☐ Feeding tube
☐ Oxygen	☐ Wound care	☐ Pressure sore prevention
☐ Comfort measures		

Psychosocial Status — Patient

☐ Emotionally stable	☐ Anxious
☐ Depressed	☐ Existential distress
☐ Not applicable	

Psychosocial Status — Family

☐ Coping well	☐ Caregiver burnout
☐ Anticipatory grief	☐ Bereavement needs identified

Action Plan / Next Steps

Issues identified	
Actions taken	
Next review date	
Reviewed by (name/role)	